SSM - Mental Health 5 (2024) 100305

Contents lists available at ScienceDirect

SSM - Mental Health

journal homepage: www.journals.elsevier.com/ssm-mental-health

Elimination of restrictive practices from acute adult mental health care services: A qualitative evidence synthesis of the lived experience literature.

S.L. Bennetts [a,b,*], G. Pepin [a,c], S. Moylan [b], R. Carolin [b], J.J. Lucas [a,c]

[a] Deakin University, Australia
[b] Change to Improve Mental Health (CHIME) Translation Research Partnership, Barwon Health & Deakin University, Australia
[c] Institute for Health Transformation, Australia

ARTICLE INFO

Handling editor: Dr A C Tsai

Keywords:
Restrictive practices
Qualitative
Review
Synthesis
Mental health care services
Reducing
Eliminating

ABSTRACT

The calls to eliminate restrictive practices (e.g., seclusion and physical restraint) from mental health care services have been gaining momentum over time and have been one of the many issues at the forefront of international mental health care systems. Whilst it is known that restrictive practices are often harmful and traumatic for the mental health service user, there is a lack of synthesis of the factors that are influencing restrictive practices' ultimate reduction and elimination from the perspective of mental health service users and practitioners. The aim of this research was to conduct a qualitative evidence synthesis of the literature regarding the perceptions and experiences of mental health service users and practitioners about restrictive practices in mental health care services. A systematic search and inclusion strategy identified 44 relevant articles for review. Inductive thematic synthesis resulted in five themes across the articles: (1) Meaning and emotional experience, (2) Re-traumatisation and dehumanisation, (3) Professional competencies and varying experiences, (4) Balancing safety versus care, and (5) Alternatives to restrictive practices. Discussion of these themes highlighted the many layered and often uncomfortable nature of restrictive practices which can pervade mental health facilities across the world.

1. Introduction

The implementation of restrictive practices in acute mental health care services has conventionally focused on managing the aggressive or agitated behaviours of service users, and to promote individual and practitioner safety (Muir-Cochrane et al., 2018). The fast-paced nature of acute mental health care services can mean that there is a limited tolerance of agitated and aggressive behaviour and thus restrictive practices are implemented as a means to coerce and control service users (McSherry and Maker, 2020). However, the problems that arise from their use, such as injuries caused, and the significant costs associated with their implementation (Abuse and Administration, 2011), has led to an international drive towards reducing and ultimately eliminating restrictive practice use and evolution over time (LeBel et al., 2014).

There are various forms of restrictive practices that exist and their definitions and interpretations vary between countries (Hem et al., 2018). Broadly, the term restrictive practices encompass practices such as chemical restraint, mechanical restraint, physical restraint and seclusion. Chemical restraint is a practice that has its own complexities and understandings in its role within acute mental healthcare settings, such as identifying what is considered chemical restraint versus other medication use (e.g., PRN), and would therefore warrant focus within its own synthesis (Muir-Cochrane et al., 2020). Thus, chemical restraint is not a focus of this evidence synthesis. Increasing over time, there has been international drive re-examining the use of the restrictive practices of restraint (physical and mechanical) and seclusion as an approach to implement more evidence-based models of care to reduce conflict, violence and the factors that contribute to the use of these restrictive practices (LeBel et al., 2014). In addition, there is little to no evidence base supporting the therapeutic need for seclusion and restraint despite their widespread use (Steinert et al., 2010). Seclusion is a control measure that involves confining a service user to a particular location for a specified amount of time in which they cannot leave freely; whereby restraint is a control measure that prevents or limits a person's freedom by either mechanical devices used for immobilisation or manual when a staff holds a patient down (Hoff, 2015). These restrictive practices are applied methodically and often considered necessary by mental health practitioners to prevent an escalation of violence and maintain safety for everyone in the clinical environment (Power et al., 2020).

Historically, the management of service users who displayed

* Corresponding author. School of Health and Social Development, Deakin University, Locked bag 20001, Geelong, VIC, 3220, Australia.
E-mail address: stephanie.bennetts@deakin.edu.au (S.L. Bennetts).

https://doi.org/10.1016/j.ssmmh.2024.100305
Received 29 August 2023; Received in revised form 22 February 2024; Accepted 22 February 2024
Available online 8 March 2024
2666-5603/© 2024 The Authors. Published by Elsevier Ltd. This is an open access article under the CC BY license (http://creativecommons.org/licenses/by/4.0/).

aggressive behaviours and were agitated was through institutionalisation (Dyck, 2013). One of the consequences of the deinstitutionalisation that occurred after World War II, included a shift from treatment of mental ill health to prevention, which transferred the responsibility of mental health services delivery to community-based services (Gooding, 2016). The result of this was that people went back into the community, but when they were very unwell, they still needed acute inpatient care. Therefore, there is an increased need to know how and when to apply restrictive practices. For example, in Australia, this change in the model of care occurred more specifically around the 1980s and 1990s (Care, 2000; Gerrand, 2005; Meadows and Singh, 2003). It was at this time that large scale psychiatric institutions were exchanged with a community mental health service approach, which involved the dispersal of authority for service provision and coercive powers to a range of entities in society, such as hospitals (Gooding, 2016). However, the complex needs of those attending these acute mental health settings have left the mental health services with a lack of resources and at a loss as to how to balance the risks and rights of the service user and practitioners' safety through the elimination of restrictive practices. This is reflected in the changeable trend in the incidence of restrictive practices in acute mental health facilities (Australian Insititute of Health and Welfare, 2022).

Governments internationally have had success in reducing rates and duration of restraint and seclusion (Department of Health Victoria, 2013; Fletcher et al., 2017; Hernandez et al., 2017; LeBel et al., 2014; Wieman et al., 2014), and continue work to understand why the use of these restrictive practices will not permanently cease. This includes examples of employing executive-level witnessing (i.e. management staff intervention) and review of restraint events as key strategies for restraint minimisation (Hernandez et al., 2017). Curie (2005) demonstrated the success of a hospital in Pennsylvania in the United States, which has virtually eliminated seclusion and restraint through adopting a change of culture, united towards the cessation of the use of restrictive practices as a treatment modality. Moreover, LeBel and Goldstein (2005), demonstrated that the elimination of seclusion and restraint not only saves money for mental health services but also improves service users outcomes and staff working conditions. International efforts are proving that not only is the reduction and elimination of restrictive practices achievable, but it also leads to additional benefits for the mental health service user, practitioners, and the service provider.

There are specific programs that are being generated and constantly empirically evaluated to determine their success in reducing and eliminating restraint and seclusion. For example, in Australia, Federal funding for implementation of evidence-based practices such as the Six Core Strategies© framework to reduce seclusion and restraint has occurred (LeBel et al., 2014; Wieman et al., 2014), along with the trial of the Safewards© Model in Victoria (Fletcher et al., 2017). Smith et al. (2015) reported that in reducing the rates and duration of restraint through initiatives such as these, there has not been an increase in assaults, which is a fear amongst those who advocate for the use of restrictive practices (Muir-Cochrane, 2018). In contrast, the Finnish medical system focuses on the need for medical treatment of service users experiencing mental ill health rather than their civil liberties (Putkonen and Vollm, 2007). These international perspectives are important examples of effectively working towards reducing and eliminating restrictive practices in acute mental healthcare services.

Further to the advancement of programs that look towards the reduction and elimination of restraint and seclusion, there is a worldwide movement towards embedding lived experience in the development of this type of reform such as those detailed in the recent Royal Commission into Victoria's Mental Health System (State of Victoria, 2021). The growing value on the use of 'expertise by experience' aspect of service design (Perkins et al., 2010) is critical when evaluating systems that impact multiple individuals. Not only does the inclusion of service user and practitioner voice lend to a greater holistic approach to balancing the needs of the individuals affected on both sides, it also ensures that power and authority regarding such sensitive topics as the elimination of restrictive practices, is equally shared (Byrne et al., 2018).

The use of restrictive practices in acute mental health services is a controversial and ongoing practice that takes place across the world. There have been some recent reviews of the literature (Butterworth et al., 2022; Cusack et al., 2018; Kodua et al., 2023), that have made important contributions in this area. However, these reviews have tended to focus on one or two types of restrictive practices, or only from the perspective of either mental healthcare service users or practitioners. Where this current synthesis of the qualitative literature is unique, is in its consideration of multiple perspectives (service users and practitioners) across multiple forms of restrictive practices (seclusion and physical and mechanical restraint). The use and proposed elimination of restrictive practices in acute mental health services from the perspective of both the mental health service user and practitioners within the one synthesis will also provide greater insight to the issue at hand. Therefore, this current synthesis is the first of its kind to investigate and integrate the perceptions and experiences of both the mental health practitioners and service users of the restrictive practices of both seclusion and restraint and would thus add a significant contribution to understanding lived experience of restrictive practice use internationally.

1.1. Aim and review question

The aim of this study was to answer the following question: "what are the perceptions and experiences of restrictive practice use and their proposed elimination in adult acute mental health services from the perspective of the mental health service user and practitioner?"

1.2. Author group positioning

The author group consists of a cross-disciplinary team (allied health, social work, occupational therapy, disability and social inclusion); all of whom are committed to promoting the health and wellbeing of mental health care service users and practitioners.

2. Material and methods

A Qualitative Evidence Synthesis (QES) is conducted when a systematic review is required of qualitative research evidence (Carroll, 2017). The synthesis included four steps: (1) systematic search and sampling of the literature, (2) methodological quality and risk of bias assessment, (3) data extraction, and (4) data synthesis (Lucas et al., 2022; Zimmer, 2006)

2.1. Search and inclusion strategy

All reviewers (SLB, JJL, GP, SM, RC) developed the search and inclusion strategy which was reviewed by one of the University Librarians not otherwise involved in the review. Specific keywords (along with their synonyms, combinations, Boolean operates, and relevant MeSh/Subject terms) were applied based upon the inclusion and exclusion criteria (Table 1). The electronic search strategy was applied to the following databases via EBSCOHost: CINAHL, MEDLINE, APAPsychInfo, as well as to EMBASE via embase.com, with no limitations applied. Translation of the key search terms and combinations were made to suit each specific database. An initial search was conducted and refined in March 2022 using only key terms (please see supplementary file for full list of search terms): ('restrictive practices*', 'restrictive intervention*', 'physical intervention*', restraint, coercion, reactive, seclusion or constraint) and (barrier* or enabler* or obstacle* or facilitator*) and ('mental health setting*' or 'mental health care setting*' or psychiat* or 'emergency department*' or 'mental health practice*') and (reduc* or eliminat*). The exact search was re-run in June 2023 to ensure all up to date articles were captured.

Table 1
Inclusion and Exclusion Criteria using the SPIDER Tool.

SPIDER Tool	Inclusion Criteria	Exclusion Criteria
S Sample1	Mental health service user[a]	Absence of mental health service user[a]
S Sample2	Mental health service practitioner [b]	Absence of mental health service practitioner [b]
PI Phenomenon of Interest	Perceptions and experiences of the use or proposed elimination of restrictive practices[c] within adult mental health services.	Perceptions and experiences unrelated to use or proposed elimination of restrictive practices[c] within adult mental health services.
D Design	Qualitative methods or mixed methods where qualitative data are explicitly collected and analysed.	Quantitative methods.
E Evaluation	Experiences directly reported from mental health service users and/or practitioners.	Only indirect reporting from mental health service users and practitioners.
R Research Type	Primary peer reviewed research. Published in English or have an English translation available.	Grey literature, abstracts, commentaries, theoretical papers or unpublished theses Publication Language: languages other than English where English translation not available.

[a] There are a number of terms employed throughout international mental health policy, legislation and literature to refer to people accessing mental health services. These terms include 'consumer', 'client', 'service user', and patient.
[b] The term 'mental health service practitioner' include people responsible for providing care for those that are experiencing mental illness within an acute mental healthcare setting and it encompasses an array of practitioners that would not historically be represented when discussing mental health practices, and encompasses those roles performed by nurses, psychologists, social workers, and physicians.
[c] Restrictive practices include physical and mechanical restraint, as well as seclusion.

Specific inclusion and exclusion criteria, using the SPIDER tool (Cooke et al., 2012; Page et al., 2021), are outlined in Table 1. The SPIDER tool is specifically utilised for qualitative research. The sample included mental health care service users and practitioners only, and their perceptions and experiences throughout the use of restrictive practices, as well as the proposed reduction and elimination of restrictive practices exclusively. This was to avoid restrictive practice research that often occurs also outside of mental healthcare services (i.e. schools and in the disability sector). In addition, only qualitative data sets were considered i.e. interview and qualitative survey data, as quantitative data would not lend itself to the richness of qualitative evaluation. No date limitations were set, however publications were to be in English and only primary peer reviewed literature would be considered.

Titles and abstracts of records from databases were screened independently by two reviewers (SLB and JJL) to exclude those not meeting the inclusion criteria. Duplicate papers were also removed. The full texts were then retrieved for all remaining records and further reviewed against the inclusion criteria and assessed for eligibility by the two same review team members (SLB and JJL). Any disagreement between them over the eligibility of particular studies was resolved through discussion and consensus. If they could not form consensus a third-party reviewer (GP) would be consulted. The online systematic review platform, Covidence, was utilised to facilitate this article selection and inclusion process and was reported according to the Preferred Reporting Items for Systematic Reviews and Meta-Analyses (PRISMA) 2020 statement (Page et al., 2021). A database re-run of searches was executed again on the June 5, 2023 to update the search and include any recently published data.

2.2. Methodological quality and risk of bias assessment

The methodological quality of the included articles was assessed using the Critical Appraisal Skills Programme CASP (2018) Qualitative Checklist (CASP-Q). For each of the ten CASP-Q appraisal questions, a score of 1 (*Yes*), 0.5 (*Unsure*), or 0 (*No*) was selected. All articles were then provided an overall quality rating as per Butler et al. (2016) recommendations: "high quality" (score: 9–10), "moderate-quality" (score: 7.5–8.5). and "low-quality" (score: <7.5). Two reviewers (SLB, GP) independently completed the CASP checklist for each included article. Any disagreements amongst the reviewers were discussed and resolved through consensus. If consensus was not reached a third reviewer (JJL) independently assessed the article and made the final decision. No article was excluded based on their overall quality rating, but rather informed the author group of the methodological strengths and limitations of the included article pool (Dixon-Woods et al., 2006). Sensitivity analysis was then conducted to assess for risk of bias attributed to methodological quality (Carroll et al., 2012).

2.3. Data extraction

Data extraction from included studies were completed via a coding sheet to collect information concerning study characteristics (for example: sample size, country, work place setting). Also, the direct participant quotes formed the first order data and the authors key themes formed the second order data that were extracted from the articles. Data extraction was undertaken predominantly by one reviewer (SB) and checked for accuracy and completeness by a second reviewer (JJL) via reflexive discussions with (SB) and using the qualitative data analysis tool, Nvivo (Version 20). Any conflicts between reviewers not resolved through consensus were done so with the support of a third reviewer (GP).

2.4. Data synthesis

The extracted data then formed the basis of the inductive thematic synthesis (Thomas and Harden, 2008; Zimmer, 2006). An inductive approach to the thematic synthesis was chosen to permit an open exploration of the available literature and the finding within and across each included article. Inductive coding of the extracted data occurred to, initially, identify the first and second order data from each article into smaller meaning units that described the different perceptions and experiences of the mental health service users and practitioners of restrictive practice use in acute mental health services. Similar codes were then grouped into larger analytical themes relating to the key areas of the perceptions and experiences of mental health service users and practitioners of restrictive practice use in acute mental health services. These analytical themes then formed the basis of the synthesis results and discussion.

3. Results

3.1. Quantity, characteristics, and quality of included articles

3.1.1. Results of screening and selection process

There were 3089 records retrieved using the search and inclusion strategy. An additional four records were retrieved through reference chaining. After duplicates were removed ($n = 1050$), title and abstract screening commenced and resulted in a further 1888 records being excluded. In reviewing the full-text of the remaining records, there were 44 articles deemed eligible for inclusion in the synthesis (Fig. 1;

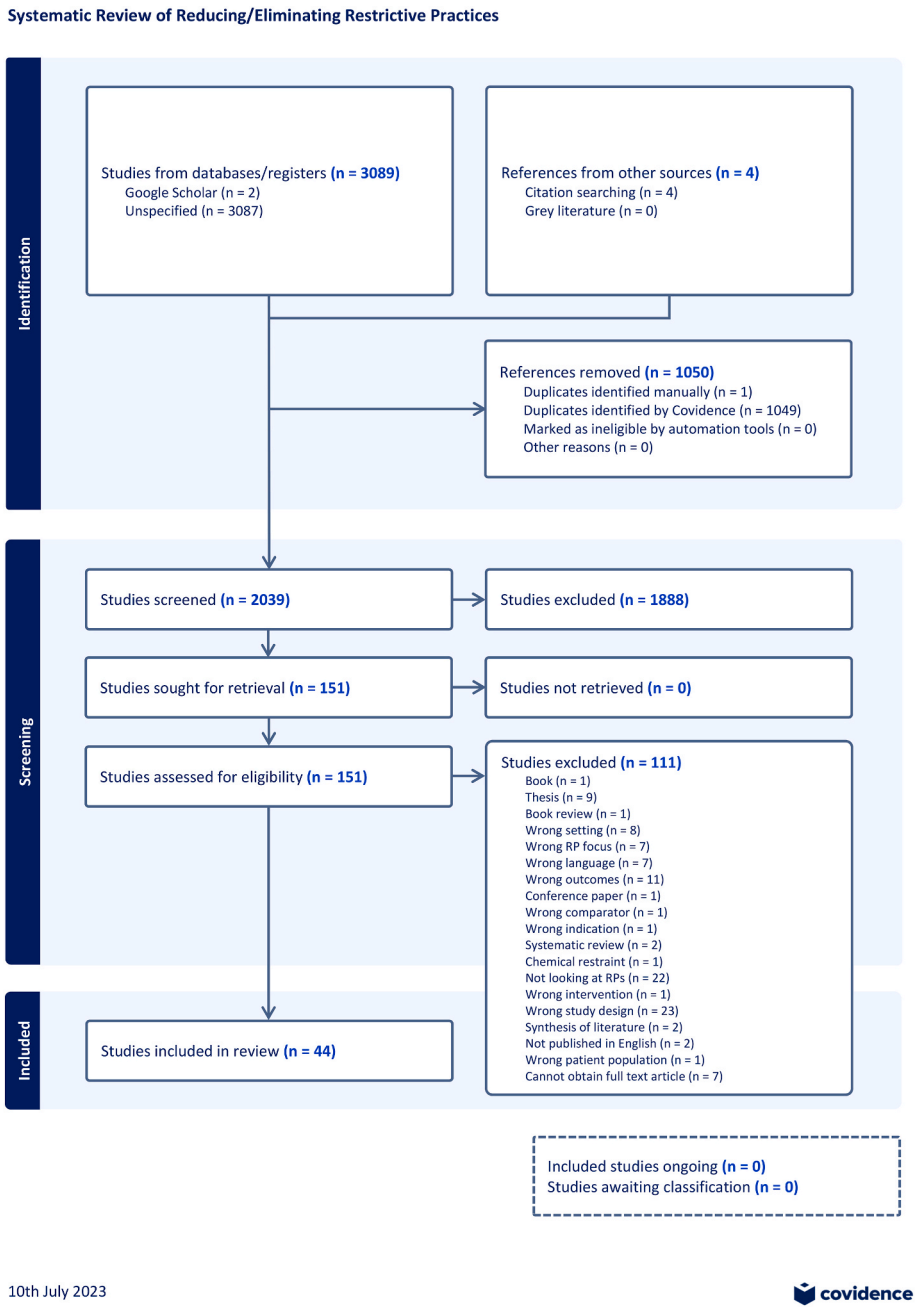

Fig. 1. PRISMA 2020 flow diagram of the article screening and selection process.

Table 2). All articles were published between 1995 and 2023. There were 43 (97.7%) qualitative articles, with the remaining one article (2.5%) reporting a mixed-methods design.

3.1.2. *Countries*

Eight articles (18%) included studies undertaken in Australia. The remaining articles reported studies from Brazil (2.27%; n = 1), Canada (11.36%; n = 5), United Kingdom (15.9%; n = 7), Finland (9%; n = 4), United States of America (9%; n = 4), China (4.5%; n = 2), New Zealand (4.5%; n = 2), Sweden (4.5%; n = 2), Iran (2.2%; n = 1), Netherlands (2.2%; n = 1), Nigeria, (2.2%; n = 1), Norway (4.5%; n = 2), South Africa (2.2%; n = 1) and Spain (4.5%; n = 2). The included article pool is northern-hemisphere dominant (25%; n = 10) and all include predominantly Western European countries or countries with a history of European colonisation.

3.1.3. *Participant ages and gender Identities*

There were approximately 339 mental health service user, and 439 practitioner voices represented across the included articles. Service users ages ranged from 14 to 70 ($SD \pm 28$) years, and practitioners ages ranged from 22 to 74 ($SD \pm 26$) years. Sixteen articles (n = 16; 42.5%) did not explicitly report the gender of the participants, and only one article specifically considered women's perspectives as mental health service users of restrictive practices (n = 1; 2.5%).

3.1.4. *Restrictive practice type and participant category*

The most common restrictive practice represented across all articles was restraint (physical/mechanical) (n = 17; 42.5%), followed closely by the reporting of both seclusion and restraint (physical/mechanical) (n = 14; 35%) and less often seclusion on its own (n = 9; 22.5%). Half of the articles focused on solely the perspectives of mental health practitioners (n = 20; 50%) compared to that of the perspectives of mental

Table 2
Summary data of included article pool (n = 44).

Study #	Author/s	Year	Country	SPIDER					Quality Score
				Sample	Phenomenon of Interest	Design	Evaluation	Research Type	
1	Allikmets et al.	2020	United Kingdom	14 patients (males)	A service evaluation appraising inpatients' perspective of processes occurring before, during and after seclusion in a psychiatric intensive care unit.	A qualitative approach	The central theme was perceived lack of communication in the patient-professional relationship, which manifested itself as (i) violence against patients, (ii) lack of psychological support, and (iii) the need for alternatives.	Q	9.5
2	Aluh et al.	2023	Nigeria	16 doctors and 14 nurses from two psychiatric hospitals in two regions of Nigeria.	To explore what mental health professionals in Nigeria think about coercion, why it is used, and what contextual factors influence its use in mental healthcare services.	Qualitative exploratory design	Coercion was viewed as being for the best interests of patients, a means to an end, and effective for achieving desired outcomes. Safety was both a reason to use a coercive measure and a deterrent to using specific coercive measures thought to be unsafe. The socio-cultural context, obsolete mental health legislation, staff shortages, and attitudes were factors influencing the use of coercion in mental healthcare.	Q	10
3	Bachman et al.	2022	Norway	18 nurse and ward staff (age range = 22–45 years)	To explore nurses' and ward staff's perceptions and assessments of patient care while implementing mechanical restraints.	Qualitative descriptive using COREQ guidelines.	The participants believed that mechanical restraints should be used as a last resort and that safeguarding patients during implementation is important; however, their assessments of the patients' physical and mental conditions varied. A clear difference emerged in how management qualified professionals handled situations prior to and during the implementation of mechanical coercive measures.	Q	9.5
4	Bigwood et al.	2008	New Zealand	7 practitioners (3 women, 4 men)	To understand the nurses experience of physical restraint.	Van Manen's descriptive hermeneutic phenomenological methodology.	The most encompassing theme that emerged from the data was: It's Part of the Job, which was constituted by the subtheme of Control. This subtheme consisted of two facets: Conflicted Nurse and Scared Nurse.	Q	10
5	Bonner et al.	2002	United Kingdom	Patients and staff (no n, gender, or age range reported)	Identify and discuss the factors that they found helpful and unhelpful during and in the immediate aftermath implementing physical restraint.	Semi-structured interviews	The patients valued staff time and attention but felt that they received too little attention. Both nurses and patients discriminated between permanent and	Q	9.5

(*continued on next page*)

Table 2 (*continued*)

Study #	Author/s	Year	Country	SPIDER					Quality Score
				Sample	Phenomenon of Interest	Design	Evaluation	Research Type	
							temporary staff. Patients reported feeling upset, distressed and ignored prior to the incidents and isolated and ashamed afterwards. Post incident debriefing was valued by all but was patchy for staff and rarer still for patients. Patients feared the possibility of being restrained. Half of the patients and several staff members reported that the incidents had reawakened distressing memories of previous traumatic events.		
6	Brophy et al.	2016	Australia	30 consumers (17 women, 13 men; age rage = 20–70)	To present the results of ten focus group discussions with people with lived experience of mental health issues and also carers, family members and support persons in relation to the use of seclusion and restraint.	Focus group discussions	Six themes emerged from the analysis, these being: human rights, trauma, control, isolation, dehumanisation and 'othering', and anti-recovery.	Q	10
7	Chien et al.	2004	China	30 psychiatric inpatients (12 women, 18 men; mean age 31.2 (5.8))	Explore the experiences and feelings of psychiatric inpatients concerning their first encounter with physical restraint.	Qualitative exploratory study	Physical restraint could be a therapeutic intervention when health professionals were able to provide psychological and informational support to patients throughout the procedure.	Q	9
8	Ezeobele et al.	2014	United States of America	20 patients (8 women, 12 men; age range = 19–53)	Explore and describe the lived experiences of psychiatric patients who were secluded at a free-standing acute care hospital located in South-western United States (US)	Phenomenological study	Findings uncovered four themes: (1) alone in the world; (2) staff exert power and control; (3) resentment towards staff; and (4) time for meditation.	Q	9.5
9	Holmes et al.	2004	Canada	6 patients ("mixed gender")	The lived experiences of psychiatric patients who were secluded at a free-standing	Phenomenological research design	Three main themes that appeared to be central to their experience of seclusion: their experience of seclusion on an emotional level, their perception of this intervention, and how they coped during their stay in the seclusion room.	Q	8
10	Jacob et al.	2019	Canada	21 nurses, 19 patients	To gain access to the bodily phenomenon of being placed in mechanical restraints.	Interpretative phenomenological analysis	The comparative analysis is presented under the following headings: (1) context of care, (2) meaning of quality of care, (3) emotional reactions and nurse–patient relationship, (4) meeting the needs and	Q	9.5

(*continued on next page*)

Table 2 (*continued*)

Study #	Author/s	Year	Country	SPIDER					Quality Score
				Sample	Phenomenon of Interest	Design	Evaluation	Research Type	
							(5) need for alternatives.		
11	Kontio et al.	2010	Finland	31 patients (63% men; age range = 20–64)	This study explored psychiatric inpatients' experiences of, and their suggestions for, improvement of seclusion/restraint, and alternatives to their use in Finland.	Qualitative method using focus groups	Patients' perspectives received insufficient attention during seclusion/restraint processes. Improvements (e.g., humane treatment) and alternatives (e.g., empathetic patient–staff interaction) to seclusion/restraint, as suggested by the patients, focused on essential parts of nursing practice but have not been largely adopted.	Q	9.5
12	Kontio et al.	2012	Finland	22 nurses and 5 physicians (52% women; age range = 26–59)	The present study explored nurses' and physicians' perceptions of what actually happens when an aggressive behaviour episode occurs on the ward and what alternatives to seclusion and restraint are actually in use as normal standard practice in acute psychiatric care.	A descriptive qualitative approach with open-ended focused interview questions	The participants believed that the decision-making process for managing patients' aggressive behaviour contains some in-built ethical dilemmas. They thought that patients' subjective perspective received little attention. Nevertheless, the staff proposed and appeared to use a number of alternatives to minimize or replace the use of seclusion and restraint.	Q	9.5
13	Lanthen et al.	2015	Sweden	10 former psychiatric patients (5 women, 5 men; age range = 32–70)	To examine psychiatric patients' experience of mechanical restraints and to describe the care the patients received.	A qualitative design with an inductive approach were used, based on interviews with patients who once been in restraints.	This study resulted in an overbridging theme: Physical Presence, Instruction and Composed Behaviour Can Reduce Discontent and Trauma, including five categories.	Q	9.5
14	Larue et al.	2010	Canada	24 nurses (15 women, 9 men; age range = <30–59)	This study explores and describes nursing interventions performed during episodes of seclusion with or without restraint in a psychiatric facility and examines the relationship between the interventions' local protocols and best-practice guidelines.	Descriptive study	Analysis of the data reveals that the interventions meet quality standards. However, nursing practice would be further refined by conducting post-event reviews, especially by discussing how a client's aggressive behaviour is to be understood.	Q	9.5
15	Ling	015	Canada	55 inpatients (33% women, 66% men)	To examine debriefing data to understand experiences before, during, and after a restraint (seclusion, chemical, and physical) event from the perspective of inpatients at a large urban mental health and addiction hospital.	Qualitative data from the forms were analysed thematically.	In patients view restraint negatively and do not experience it as a therapeutic intervention	Q	9

(*continued on next page*)

Table 2 (*continued*)

Study #	Author/s	Year	Country	SPIDER					Quality Score
				Sample	Phenomenon of Interest	Design	Evaluation	Research Type	
16	Manderius et al.	2023	Sweden	12 psychiatric mental health nurse (10 women, 2 men)	The aim of this study is to shed light on psychiatric mental health nurses' ethical considerations and on the factors influencing them when performing coercive measures	Qualitative interview study	The study revealed a duality that created two categories: Ethical considerations that promote the patient's autonomy and health and Obstacles to ethical considerations. Based on this duality, ethical considerations were made when performing coercive measures to alleviate suffering and promote health. The result shows a high level of ethical awareness in clinical work. However, a request emerged for more theoretical knowledge about ethical concepts that could be implemented among the staff.	Q	9.5
17	Manzano-Bort et al.	2021	Spain	31 nurses (17 women, 14 men)	To explore mental health nurses' experiences whilst managing a patient with psychomotor agitation, and the factors that influence the decision to use mechanical restraints.	A qualitative study design with a hermeneutical approach was developed.	Four themes emerged from the analysis: 1) Nurses' perceptions of restraint methods, 2) Factors influencing decision-making, 3) Consequences for professionals of the use of mechanical restraint and 4) Alternatives to mechanical restraint.	Q	9.5
18	Martello et al.	2018	Canada	6 nurses (83.6% women)	The purpose of this study was to explore nurses' perceptions of engaging with patients to reduce the use of restrictive practices in an inpatient psychiatric unit.	A qualitative-descriptive study.	Four major themes emerged from the data analysis: unit engagement practices, managing the escalation by engaging with patient, engaging during the use of restrictive practices, and factors influencing engaging with patient to reduce restrictive practices.	Q	9.5
19	Mayers et al.	2010	South Africa	43 service users	In a collaborative two-phase study between mental health care providers and mental health service users, the perceptions and experiences of a group of service users who have been exposed to sedation, seclusion and restraint were explored.	Participatory approach.	There is a need to humanize service users' experiences during episodes of acute illness.	Q	9.5
20	Meehan et al.	2000	Australia	12 patients (5 women, 7 men; age range = 18–52)	Perceptions of seclusion.	Semi-structured interviews	Five major themes emerged: use of seclusion, emotional impact, sensory deprivation, maintaining control and staff ± patient interaction	Q	9.5
21	McCain & Kornegay	2005	United States of America	8 seasoned psychiatric nurses (5 women, 3 men)	This phenomenological study examined how eight seasoned psychiatric nurses	Phenomenological study	Eight themes emerged from the analysis of the nurses' experiences that	Q	10

(*continued on next page*)

Table 2 (*continued*)

Study #	Author/s	Year	Country	SPIDER					Quality Score
				Sample	Phenomenon of Interest	Design	Evaluation	Research Type	
					experienced the therapeutic use of physical behavioural health restraint in their current practice settings.		described the beliefs that use of behavioural health restraints should be resorted to only after less restrictive measures, safely implemented, failed to control potentially harmful behaviour, avoided through early recognition and the subsequent implementation of less restrictive interventions.		
22	Moghadam et al.	2013	Iran	14 nurses (age range = 25–52)	To investigate the experiences of nurses working in psychiatric wards regarding physical restraint.	Semi-structured interviews	Four categories emerged: (1) Restraint as a multi-purpose procedure, (2) Processing of physical restraint, (3) Restraint as a challenging subject and (4) The effects of restraint on the spectrum.	Q	9
23	Mooney & Kanyeredzi	2021	United Kingdom	8 staff (3 women, 5 men)	To explore psychiatric healthcare staff experience of RP on inpatient wards in the UK.	Semi-structured interviews	Three themes were identified: the coexistence of accountability, power and subjection; impacts on the individual and professional relationships and coping with difficult experiences and emotions. Restrictive practice can negatively affect staff experience, working relationships, and wellbeing.	Q	9.5
24	Moran et al.	2009	Finland	23 nurses (15 women, 8 men)	This study specifically explored the emotions and feelings experienced by a group of psychiatric nurses working in Ireland in relation to incidents of restraint and seclusion.	Qualitative research approach	Three themes were created consisting of: (1) the last resort – restraint and seclusion; (2) emotional distress; and (3) suppressing unpleasant emotions.	Q	9.5
25	Muir-Cochrane & Harrison	1996	Australia	7 nurses qualified in general and psychiatric nursing	A grounded theory study of psychiatric nurses' use of seclusion in two closed wards in a metropolitan teaching hospital in Australia.	Grounded theory methodology	"Intervening" and "mainstreaming" are the categories that describe nursing interventions developed from constant comparison of the data collected. Details of these interventions are presented, with reference to the skills that participants describe as necessary in the care of a mentally ill patient being secluded and then re-integrated into ward life.	Q	9.5
26	Muir-Cochrane	1996	Australia	7 psychiatric nurses	This paper examines the use of seclusion on closed psychiatric wards	Grounded theory methodology	Whilst expert therapeutic intervention were	Q	8.5

(*continued on next page*)

Table 2 (*continued*)

Study #	Author/s	Year	Country	SPIDER					Quality Score
				Sample	Phenomenon of Interest	Design	Evaluation	Research Type	
					in the management of acutely disturbed patients.		described by clinicians, they are contextualised within a framework of power and control - a framework that stands in stark contrast to contemporary philosophies of nursing care, providing impetus for a reconsideration of the use of constraining practices in the care of mentally ill people.		
27	Muir-Cochrane et al.	2015	Australia	39 mental health nurses	To explore nurses' experiences of restraint and seclusion in short-stay acute old age psychiatry inpatient units and how theseexperiences underpin resistance to eliminating these practices.	Interpretative phenomenological analysis	The results provide one overarching theme, lack of accessible alternatives to restraint and seclusion, indicating that nurses believe there are no effective, accessible alternatives to these practices. Three related themes contribute to this perception. First, an adverse interpersonal environment contributes to restraint and seclusion, which relates to undesirable consequences of poor staff-to-patient relationships. Second, an unfavourable physical environment. contributes to aggression and restraint and seclusion use. Third, the practice environment influences the adoption of restraint and seclusion.	Q	9
28	Muir-Cochrane et al.	2018	Australia	20 mental health nurses (11 women, 9 men; age range = 25–74)	The aim of this qualitative study was to explore mental health nurses' views of the potential elimination of restrictive practices.	Qualitative descriptive study	Overall, the nurses expressed significant fear about the potential elimination of restrictive practices and saw themselves as being blamed for both the use of these practices and the consequences should they be eliminated. Findings detail the conflicts facing staff in balancing the need for ward safety for everyone present while at the same time providing person-centred care.	Q	9.5
29	Olofsson & Norberg	2001	Sweden	Seven triads of patient, nurse and physician narrated their experiences of the same coercive event. The 21 interviews were	The aim of the study was to increase understanding of psychiatric patients', nurses' and physicians' experience of coercion, in relation to their own	Descriptive-explorative qualitative study	The salient aspect described by all three parts was the importance of human contact and having a mutual relationship. These findings indicate a need for	Q	9.5

(*continued on next page*)

Table 2 (*continued*)

Study #	Author/s	Year	Country	SPIDER					Quality Score
				Sample	Phenomenon of Interest	Design	Evaluation	Research Type	
				analysed focusing on narrative elements.	and the other parties' experiences.		more dialogue between patients and staff. The dialogue should aim at making staff understand patients' feelings better regarding coercion and at informing the patients about the coercive measures.		
30	Perez-Torbio et al.	2022	Spain	10 Spanish mental health nurses (8 women, 2 men; age range = 23–41)	To describe the experiences of mental health nurses who have used mechanical restraint in practice.	Qualitative descriptive methodology	Three main themes arose from the analysis of interview transcripts, (i) symmetrical trauma, (ii) moral injury and (iii) broken trust.	Q	9.5
31	Sambrano & Cox	2013	Australia	3 Indigenous people	To explore how three Indigenous people experienced the controversial practice of seclusion.	Hans-Georg Gadamer's phenomenology guided analysis	Participants viewed seclusion negatively: police involvement in psychiatric care; perceptions of being punished and powerless; occasions of extreme use of force; and lack of care were prominent themes throughout the interviews.	Q	10
32	Scholes et al.	2022	United Kingdom	20 women accessing inpatient mental health services	The aim of this paper was to explore women's experiences of restrictive interventions within UK inpatient mental health services.	Qualitative research design	Three primary themes were reported from women's experiences: (1) powerlessness, (2) dehumanisation, and (3) relationships and communication.	Q	9.5
33	Sequiera & Halstead	2002	United Kingdom	14 inpatients of a secure psychiatric hospital (age range = 14–51)	This study examines the experiences of physical restraint procedures reported by in-patients of a secure mental health service.	Qualitative (inductive) methodology	Patients had differential experiences of similar physical procedures. Most reported some negative psychological experience of restraint. Anger and anxiety were two major themes. Some respondents held the perception that restraint was used to punish patients and several suggested that restraint incited further violence and aggression. Some female service users reported that restraint evoked flashbacks of previous sexual trauma.	Q	10
34	Sequiera & Halstead	2004	United Kingdom	17 nursing staff (8 women, 9 men; age range = 18–50)	The study examines the experiences of physical restraint procedures reported by nursing staff in a secure mental health service.	Qualitative (inductive) methodology	Nursing staff reported a range of emotional responses to the use of restraint procedures. They included anxiety, anger, boredom, distress and crying.	Q	9.5
35	Smith et al.	1995	United States of America	4 case studies (3 women, 1 men)	This study explores how patients with a history of rape perceive the experience of being placed in restraints while being treated at an	Interviews	During a group meeting for manic depressive and depressed clients, several individuals compared their	Q	2.5

(*continued on next page*)

Table 2 (*continued*)

Study #	Author/s	Year	Country	SPIDER					Quality Score
				Sample	Phenomenon of Interest	Design	Evaluation	Research Type	
					inpatient psychiatric unit.		experience of being in leather restraints to their experience of rape. Follow up interviews were done with their permission.		
36	VanDerNagel et al.	2009	Netherlands	8 nurses (4 female, 4 male; age range = 24–56)	This study describes personal experiences of nurses throughout the seclusion process.	Semi-structured interviews	The emotions which came to surface in semi-structured interviews with 8 nurses were categorized in three main themes (Tension, Trust and Power) and a stress response curve was identified in the seclusion process, with specific feelings in each phase.	Q	9
37	Varpula et al.	2020	Finland	32 nurses (59% women; age range = 23–60)	The aim of this study is to describe the risks for occupational hazards in patient seclusion and mechanical restraint practices as well as ideas for improvement identified by nurses.	Qualitative descriptive design was adopted, using focus groups	Four themes of risk for occupational hazards were identified: patient-induced, staff-induced, organization-induced, and environment-induced risks.	Q	10
38	Vedana et al.	2018	Brazil	29 nurses (14 women, 15 men; age range = 27.5–65)	In this qualitative study, we aimed to understand the experiences and perceptions of nursing staff regarding physical restraint in psychiatric units.	Qualitative study using symbolic interactionism (SI) as a theoretical framework.	Physical restraint was considered unpleasant, challenging, risky, and associated with dilemmas and conflicts. The nursing staff was often exposed to the risks and injuries related to restraint. Professionals sought strategies to reduce restraint-related damages, but still considered it necessary due to the lack of effective options to control aggressive behavior.	Q	10
39	Wai-Tong & Lee	2007	China	15 psychiatric nurses (age range = 30–56)	The purpose of this study were to identify registered psychiatric nurses' knowledge, attitudes and practice issues regarding the use of physical restraints in Hong Kong, and to examine the factors influencing nurses' decisions to use restraints on their patients.	Descriptive exploratory study	Four major themes were identified: (1) attitudes towards the use of a safety measure and effective intervention for patients, (2) insufficient consideration of alternative measures and/or adverse consequences, (3) ethical dilemmas, and (4) psychological reactions towards restraint use.	M-M	10
40	Wharewa-Mika et al.	2016	New Zealand	16 maori participants with high levels of clinical, cultural and consumer expertise.	This qualitative study explored Māori clinical, cultural and consumer perspectives on potential strategies and initiatives considered likely to facilitate prevention of, and reduction in, the use of seclusion, with tāngata whai i te ora (Māori	A hui (gathering) over 2 days was held with 16 Māori participants	A thematic analysis of the hui data generated three key categories: Te Ao Māori (access to a Māori worldview); Te Ao Hurihuri (transforming practice); and Rangatiratanga (leadership, power, and control).	Q	10

(*continued on next page*)

Table 2 (*continued*)

Study #	Author/s	Year	Country	SPIDER					Quality Score
				Sample	Phenomenon of Interest	Design	Evaluation	Research Type	
					mental health service users) in mental health inpatient services.				
41	Wilson et al.	2017	United Kingdom	13 inpatients (7 women, 6 men; age range = 18–65) and 22 staff members (age range = 20–50s; 15 females, 7 males)	To explore mental health patients' and staff members' suggestions for reducing physical restraint	The study was grounded in a realist epistemological framework.	Findings centred on four overarching themes: improving communication and relationships between staff/patients; making staff-related changes; improving ward environments/spaces; and having more activities. However, concerns were raised around practicalities/ feasibility of their implementation.	Q	9.5
42	Wong et al.	2022	United States of America	25 patients (17 women, 8 men; age range = 18-$>$55)	This qualitative study aims to use a systems-based approach, which considers the larger context and system of healthcare delivery, to identify sociotechnical, structural, and process-related factors leading to agitation events and physical restraint use in the ED.	Grounded theory.	Three primary themes with implications for systems-based practice of agitation events in the ED emerged: (1) pathways within health and social systems; (2) interpersonal contexts as reflections of systemic stressors on behavioural emergency care and (3) systems-based and patient-oriented strategies and solutions.	Q	9
43	Wynaden et al.	2002	Australia	8 mental health nurses and 1 doctor (1 woman, 7 men)	The objective of this study is to provide new knowledge and understanding of the decision making used throughout the seclusion process.	Descriptive exploratory study	The results demonstrate that seclusion is initiated only when all other less restrictive patient management strategies have proven to be unsuccessful with the patient.	Q	9
44	Wynn	2004	Norway	12 psychiatric in patients (3 women, 9 men; age range = 21–60 years)	Twelve psychiatric inpatients were qualitatively interviewed after physical restraint.	Qualitatively interviewed	While some felt that the use of restraint had been warranted, others were more critical. Several believed that the use of restraint did protect them from hurting themselves or others. Some felt angry, fearful, and distrustful of staff after restraint, and some believed it had damaged the provider – patient alliance. Patients that had psychotic symptoms during restraint were more under-standing of staff's decision to restrain	Q	8

health service users (n = 15; 35%), and a far less portion of articles combined both perspectives (n = 5; 12.5%). Interestingly, of those mental health practitioner perspectives, the nursing profession was by far the most represented in 24 articles (60%), followed by 4 articles (10%) including physician perspectives, and only one article (2.5%) representing the combined perspectives of occupational therapists, ward managers, health assistants, social educators, and nurse assistants. Finally, there was one article (2.5%) that used blanket terms such as "staff" when referring to their mental health practitioner participants.

3.1.5. Quality and risk of bias assessment

In using the CASP-Q, which contained 10 appraisal questions, the two reviewers (SLB and JJL) assessed 440 criteria over the 44 included articles independently and agreed on 357 items (89.25%). The majority of articles (n = 44; 93%) were rated 'high' quality, with two articles (4.5%) of 'moderate' quality and one article (2.2%) of 'low' quality. The 'low' quality article was rated 'very low' quality (CASP-Q score ≤4.5) with a score of 2.5 (Smith, 1995). The primary limitation across all the articles, regardless of their overall quality rating, was in their adequately addressing the relationship between researcher/s and participants.

3.2. Inductive thematic synthesis

Through the process of inductive thematic synthesis, a total of five overarching themes were developed with corresponding sub-themes as outlined in Table 3. The first three themes begin with detailing the experiences around restrictive practices, whilst the latter two themes are more concerned around the operational aspects of restrictive practices. They are as follows: (1) Meaning and emotional experience, (2) Re-traumatisation and dehumanisation, (3) Professional competencies and varying experiences, (4) Balancing safety versus care, and (5) Alternatives to restrictive practices. There are three themes that pertain to the experiences and perceptions of both the service users and practitioners (meaning and emotional experience; balancing safety versus care and alternatives to restrictive practices), and there are two themes that pertain to the individual participant cohorts (this being re-traumatisation and dehumanisation for the service users; and professional competencies and varying experiences for the practitioners The themes are outlined in Table 3. It is important to note that these themes are not considered mutually exclusive, but rather interdependent – each having an influence on the other.

3.2.1. Theme 1: meaning and emotional experience

The meaning and emotional experiences of restrictive practices theme relates to how the service users and practitioners process, cope and convey their emotional experience of restrictive practices. This theme was reported across 19 articles (VanDerNagel et al. (2009); Bonner et al., 2002; Brophy et al., 2016; Chien et al., 2005; Holmes et al., 2004; Jacob et al., 2019; Caroline Larue et al., 2010; Manzano-Bort et al., 2021; Mayers et al., 2010; McCain and Kornegay, 2005; Meehan et al., 2000; Moran et al., 2009; Muir-Cochrane, 1996; Sambrano and Cox, 2013; Sequeira and Halstead, 2002; Sequeira and Halstead, 2004; VanDerNagel et al. (2009); Vedana et al., 2018; Wai-Tong and Lee, 2007; Wilson et al., 2018). This theme was reflected across three sub-themes: (1) Therapeutic and non-therapeutic impacts of restrictive practices, and (2) Coping with strong emotional reactions and, (3) The mental health practitioner-service user relationship.

3.2.1.1. Meaning and emotional experiences of restrictive practices as "therapeutic" versus "non-therapeutic". The therapeutic impacts of restrictive practices largely relate to positive experiences that are a result of the implementation of restrictive practices. This includes, feelings of safety and trust (Chien et al., 2005) reported by service users, and as at times necessary, as detailed by practitioners (Muir-Cochrane, 1996). However, the non-therapeutic impacts of restrictive practices reflected more so on the emotional distress Mayers et al. (2010) in which restrictive practices cause for both the mental health service user and practitioners. The anger (Sequeira and Halstead, 2002), anxiety (Sequeira and Halstead, 2002) and fear (Bonner et al., 2002) perpetuated by restrictive practices was reported vastly by practitioners and the overwhelming feelings of powerlessness and loss of autonomy was reported by service users (Chien et al., 2005). Furthermore, Brophy et al. (2016) detailed the experiences and impact of restrictive practices on services users, including describing the practices as not only isolating but that they are "anti-recovery" (p. 4). It was described by one service user in the study as:

> "Seclusion and restraint, the very practices themselves, are sort of very anti-recovery … [Recovery is] all about self-responsibility, self-direction, and then seclusion and restraint is all about someone else's control, so it doesn't actually sit with recovery at all." (p. 6)

3.2.1.2. Coping with strong emotional reactions. Coping with strong emotional reactions was a subtheme related to the individual's ability to navigate the extensive feelings around experiencing restrictive practices. McCain and Kornegay (2005), detailed how restraint was a difficult experience for the service users and practitioners involved in the experience. A nurse conveyed this through: "My experience with physical restraint has made me more tolerant of bad behaviour because physical restraint is very hard on everyone concerned." (p.240) On the contrary, Sequeira and Halstead (2004) found that practitioners reported abuse of interventions, and that although no-one was intentionally hurt, participants reported that they experienced the thought of being able to cause harm and the guilt associated with this.

3.2.1.3. The mental health practitioner-service user relationship. The mental health practitioner-service user relationship sub-theme relates to the effects on their relationship when restrictive practice intervention had been used. Jacob et al. (2019) reported that the mental health practitioner-service user relationship suffered after the implementation of a restrictive practice towards a service user, and the importance in reconciling this relationship proves to be an important task so as to provide quality mental health care. This was described within their article by a service user as:

Table 3
Table of themes.

#	Overarching Theme	Mental Health Service User	Mental Health Practitioner
1	**Meaning and emotional experience**	Meaning and emotional experiences of restrictive practices as "therapeutic" and "non-therapeutic" Coping with strong emotional reactions The mental health practitioner-service user relationship	Meaning and emotional experiences of restrictive practices as "therapeutic" and "non-therapeutic" Coping with strong emotional reactions The mental health practitioner-service user relationship
2	**Re-traumatisation/De-humanisation**	Emotional impact as a result of restrictive practices De-humanisation and power dynamics	
3	**Professional competencies and varying experiences**		Confident decision-making Delivering quality care Seeking practitioner support
4	**Balancing safety versus care**	Early intervention Physical and interpersonal environment	Early intervention Physical and interpersonal environment
5	**Alternatives to restrictive practices**	What alternatives to restrictive practices have been proposed? Continuous assessment and insufficient consideration of alternative measures	What alternatives to restrictive practices have been proposed? Continuous assessment and insufficient consideration of alternative measures

> "Well after that, it was obviously awkward. I didn't want to talk; I was nervous to talk to that nurse. I'm like oh my gosh she is […] I felt like oh she probably despises me at this point. Like I didn't want her to be my nurse ever again because I'm just like I caused this huge scene while I didn't feel like I caused it. Like maybe both of us caused it right she wasn't listening to me at the time to help me and then that's how I blew up." (p.1019)

3.2.2. Theme 2: Re-traumatisation/de-humanisation

The theme of re-traumatisation and dehumanisation refers to the experiences of the service user that details the harmful effects and consequences of the use of restrictive practices. This theme was reported across eleven articles (Bonner et al., 2002; Chien et al., 2005; Ezeobele et al., 2014; Holmes et al., 2004; Lanthén et al., 2015; Mayers et al., 2010; Olofsson and Norberg, 2001; Sambrano and Cox, 2013; Scholes et al., 2022; Sequeira and Halstead, 2002; Wharewera-Mika et al., 2016).This theme was broken down into their two categories: (1) Emotional impact of restrictive practices, (2) Dehumanisation and power dynamics.

3.2.2.1. Emotional impact as a result of restrictive practices. Emotional impact as result of restrictive practices was categorized by the psychological consequences of the use of restrictive practices for some mental health service users. These consequences resembled experiences commonly seen in instances of Post-Traumatic Stress Disorder (PTSD) (Zayfert, 2012). For example, Bonner et al. (2002) participants described the use of restraint as re-traumatising as the experience brought back memories of previous violent incidents such as rape and childhood abuse. Moreover, service users showed indicators of avoiding trauma, such as by avoiding seeking mental health help as they feared the use of restrictive practices and the consequences of their effects (Bonner et al., 2002). Holmes et al. (2004) described the powerful emotions experienced in the Seclusion room. Feelings of abandonment, rejection and deep distress (Holmes et al., 2004) were characterised by the mental health service users. One service user in Holmes et al.'s, (2004) study described this distress and abandonment:

> "In the isolation room, I cry, I bang on the door, I'm like that … I take my eyeglasses, I break them. I said to myself, if I am often like this, they will see, they will see that there is something wrong with me ….I want to talk." (p. 571).

3.2.2.2. Dehumanisation and power dynamics. Dehumanisation and power dynamics described the mental health service user's powerlessness and feelings of having their human rights violated. Scholes et al. (2022) discussed participants feeling as if they were treated as an animal. One participant described this as: "they are watching me, and they are looking at me like I'm from the zoo. Not a human being." (p. 20)

In addition to this, (Mayers et al., 2010) detailed a lack of respect for basic human dignity as some service users reported instances of provocation and teasing from practitioners. Participants across two separate studies described the implementation of restrictive practices as punishment as opposed to a protective intervention (Mayers et al., 2010; Sambrano and Cox, 2013). Furthermore, the use of restrictive practices as a means of control was reported by a service user t in the study conducted by (Sambrano and Cox, 2013) as: "You aren't allowed to raise your voice for any matter, or you get locked up in isolation … I mean everyone has bad days." (p.525)

3.2.3. Theme 3: professional competencies and varying experiences

The theme of professional competencies and varying experiences of restrictive practice intervention relates to how the mental healthcare practitioner details their personal experiences of restrictive practices, in both the professional and emotional sense. This theme was reported across sixteen articles (Bachmann et al., 2022; Bonner et al., 2002; Fereidooni Moghadam et al., 2014; Jacob et al., 2019; Manderius et al., 2023; Manzano-Bort et al., 2021; Martello et al., 2018; McCain and Kornegay, 2005; Mooney and Kanyeredzi, 2021; Muir-Cochrane and Harrison, 1996; Olofsson and Norberg, 2001; Perez-Toribio et al., 2021; Sequeira and Halstead, 2004; Varpula et al., 2020; Wilson et al., 2018; Wynaden et al., 2002). This theme captured within the mental health practitioners' perspectives was broken down into four separate sub-themes: (1) Factors influencing decision-making, (2) Meaning of quality care, (3) Practitioner support and (4) The last resort.

3.2.3.1. Factors influencing decision-making. The factors influencing decision-making around restrictive practices was significant as it detailed the complexities around the decision-making process involved in restrictive practice use. For example, Wynaden et al. (2002) detailed this as reviewing the contextual conditions and "at risk" behaviours and the use of a management hierarchy, as part of assessing the situation leading up to restrictive practice use. Martello et al. (2018) further expanded on the importance of getting to know the patient, but also added the importance on recognizing escalation and supporting the patient to express their concerns. This was done through reorientating the patient with hope that they recognize their own behaviour. Perhaps the most noteworthy factor for influencing decision-making around restrictive practice intervention was detailed in Muir-Cochrane's (1996) study which highlighted the importance of nurse practitioners' ability to be able to consistently intervene and prevent escalation of behaviours. The participants described the importance of this skill set particularly when on closed-ward environments, as this for example, enabled practitioners to offer counselling to the patient and maintain ward safety (Muir-Cochrane, 1996).

3.2.3.2. Meaning of quality care. Meaning of quality care detailed what delivery of quality care was to mental health practitioners. For example, Jacob et al. (2019) discussed practitioner quality care as being linked to the respect for protocols or procedures. When faced with the use of restrictive practices being a debate entrenched in ethical dilemma, the practitioner view was justified by the result in which it delivered, meaning how restrictive practice was used to control behaviour (Jacob et al., 2019). Varpula et al. (2020) added that proper preparation for seclusion and restraint, availability of safety equipment, adequate department resources and staffing, all contribute to greatly reducing occupational risks associated with restrictive practices. Furthermore, McCain and Kornegay's (2005) study further supported that the use of appropriate restrictive practice protocols and procedures added to monitoring and maintaining safety and thus delivery of quality of care.

3.2.3.3. Practitioner support. Practitioner support describes what mental health practitioners say they need in order to be well-supported in their workplace. Bachmann et al. (2022) discuss the lack of awareness amongst practitioners regarding restrictive practice guidelines and how to check on patients whom have undergone mechanical restraint as problematic in mental health. Bonner et al. (2002) discussed how good teamwork, communication and training as crucial to managing and sustaining a safe inpatient ward. In addition, they also highlighted the importance of policies and guidance in decision-making in respect to managing escalating behaviours and critical incidences (Bonner et al., 2002). Encouragingly, McCain and Kornegay (2005) found that the mental health practitioners in their study reported the use of restrictive practice as having greatly decreased. This was attributed to increased education, crisis prevention/intervention training and recent policy changes to enhance practitioner support (McCain and Kornegay, 2005).

3.2.3.4. The last resort. Four articles (Bachmann et al., 2022; Bonner et al., 2002; McCain and Kornegay, 2005; Moran et al., 2009) reported that restrictive practices be implemented as a safety measure and only a last resort. Practitioners echoed sentiments throughout these articles

about both seclusion and restraint practices being of necessity at times to protect the client from hurting themselves or others. A nurse practitioner described in McCain and Kornegay (2005) that: "There are certainly times when physical restraint is an appropriate intervention to use, but it should be used as a last resort." The practitioner participants in Moran et al.'s (2009) study detailed a progressive methodical process of moving from the least restrictive form of intervention whilst maintaining open communication and a calm environment. Furthermore, the participants of this study described the emotional distress they experienced when turning to restrictive practices as a 'last resort' management.

3.2.4. Theme 4: balancing safety versus care

The balancing of safety theme refers to balancing the safety of the ward environment and safety and care of the service user. Balancing safety versus care was reported in fourteen articles (Aluh et al., 2023; Bigwood and Crowe, 2008; Bonner et al., 2002; Brophy et al., 2016; Jacob et al., 2019; Ling et al., 2015; Manzano-Bort et al., 2021; Muir--Cochrane et al., 2018; Olofsson and Norberg, 2001; Scholes et al., 2022; Sequeira and Halstead, 2004; Wilson et al., 2018; Wong et al., 2022; Wynaden et al., 2002).This theme broadly relates to two sub-themes: (1) Early intervention and (2) Physical and interpersonal environment.

3.2.4.1. Early intervention. This sub-theme relates to controlling and maintaining the safety of all persons in the ward environment to provide behaviour de-escalation. For example, practitioner participants across these articles described how the implementation of early intervention (McCain and Kornegay, 2005) such as diverting the service users attention to the TV room (Bachmann et al., 2022) was important in avoiding the use of restrictive practice intervention. Furthermore, creating a manageable environment meant creating awareness of potentially aggressive behaviours such as verbal outbursts (i.e., yelling at practitioners or other service users). Encouraging the use of relaxation is another example of a de-escalation technique used to avoid implementing restrictive practices in this instance.

The physical and aesthetics of the ward environment was discussed by the mental health service users as an important part of creating a manageable environment. The participants described how the "fishbowl" ward designs often further reinforced the separation and disconnection of the service users from the practitioners and greater ward (Brophy et al., 2016). Moreover, both the practitioner and service users identified improvement of the physical/aesthetic environments on wards as a means to reduce restrictive practices. One participant described a "subtle morgueness" (Wilson et al., 2018) feeling on ward, and thus that this detracted from the recovery-orientated environment desired in an acute mental health care service.

3.2.4.2. Physical and interpersonal environment. Compounding the environment for those participants, with a less than desirable ward physical atmosphere, is the pre-cursor to the event of a restrictive practice intervention. Both the practitioner and service user participants in Bonner et al.'s (2002) study, detailed how overcrowded wards contributed to a greater likelihood of aggressive behaviour that would induce restrictive practices. Jacob et al. (2019) detailed the interdependence of the physical and interpersonal factors of the environment, such as admitting service users to rooms with no windows and lights, and how the physical environmental surroundings influence people's and behaviour within the wards. Moreover, getting to know the service users was discussed as a successful antecedent to de-escalation and prevention of restrictive practices (Wilson et al., 2018). This was further described by a staff participant as:

> "we used to use a book called All About Me – its almost like a passport – so when people arrive straight away someone will know and you can show them, people can know you like two sugars in your coffee and don't give you baked beans …" (staff member 7, eyewitness p. 193)

3.2.5. Theme 5: alternatives to restrictive practices

The alternatives to restrictive practices theme discuss the various alternative approaches to restrictive practices, as determined by the service users and practitioners. This theme was reported across nineteen articles (Allikmets et al., 2020; Bachmann et al., 2022; Bonner et al., 2002; Jacob et al., 2019; Kontio et al., 2012; Kontio et al., 2010; Larue et al., 2013; C. Larue et al., 2010; Larue et al., 2010; Martello et al., 2018; Muir-Cochrane, 1996; Muir-Cochrane and Harrison, 1996; Muir-Cochrane et al., 2018; Muir-Cochrane et al., 2015; Olofsson and Norberg, 2001; Vedana et al., 2018; Wai-Tong and Lee, 2007; Wharewera-Mika et al., 2016; Wilson et al., 2018; Wynaden et al., 2002) and was broken into two further sub themes: (1) What alternatives to restrictive practices have been proposed and, (2) Continuous assessment and insufficient consideration for alternative measures.

3.2.5.1. What alternatives to restrictive practices have been proposed. Alternatives to restrictive practices is a sub-theme that related to both the mental health service user and practitioners' suggestions of alternatives to the use of seclusion and restraint. This included strategies to reduce the need for this type of intervention and ways to transform practice. For example, in the study conducted by Kontio et al. (2012), mental health service users determined four categories of alternatives to seclusion and restraint. These including empathetic patient-staff interaction, participation in meaningful activities, therapeutic community, and biological treatments. One service user described an example of empathetic patient-staff interaction as: "… I need a human being beside me. I want to talk about my fears with the physician and nurse. I like to have a connection to them, now they are in a hurry all of the time". (p.21). While that empathetic connection was of importance to the mental health service user, for the mental health practitioner, especially that of the nursing assistants in Vedana et al. (2018) study, the use of early intervention particularly with restraint, was seen as a means to avoid injuries to staff. Interestingly, Wharewera-Mika et al. (2016) found that the service users discussed the need for a cohesive approach to changing practice, addressing these challenges and implementing change as a means to transforming practice together and eventually eliminating restrictive practices.

3.2.5.2. Continuous assessment and insufficient consideration for alternative measures. Continuous assessment and insufficient consideration for alternative measures to restrictive practices related to both the mental health practitioners and service users detailing that restrictive practices are often a means to an end when it comes to protecting service users from themselves, others and the practitioners (Olofsson and Norberg, 2001). Vedana et al. (2018) also described the lack of knowledge by staff of alternatives for the management of aggressive behaviours and a greater inconsistencies and lack of cohesion when it came to the restrictive practice of restraint.

In addition, Jacob et al. (2019), found that the service users reported alternatives to restrictive practices that impede less on their freedoms and focused more on de-escalation techniques. Furthermore, Muir-Cochrane et al. (2015) found that the physical environment played a big role in influencing the use of restrictive practices. They also found that low-stimulation rooms, access to garden space and alternative spaces for patients to access at their own accord were just as effective as seclusion and restraint (Muir-Cochrane et al., 2015)

4. Discussion

The purpose of this article was to synthesise the perceptions and experiences of restrictive practices and the prospect of their elimination from the perspective of mental health service users and practitioners reported in the qualitative research. This qualitative evidence synthesis explores the literature spanning almost 30 years; however, the research of lived experience and restrictive practices has only been more

noticeably gaining traction within the literature in the last 5–10 years. Moreover, the perspective is often vastly representative of the practitioner population only. Notably, the story has not changed much over that time. Practitioners feel they need restrictive practices in order to maintain control of the ward environment, and service users feel that restrictive practice use contributes to their feelings of lack of safety and violation of human rights.

Overall, the diversity of experiences was focused on balancing safety versus care, the meaning and emotional experiences of restrictive practices, and discussion of alternatives to restrictive practices. Re-traumatisation and dehumanisation felt by mental health service users, and professional competencies and varying experiences of mental health practitioners were prominent perspectives from both participant groups. Interestingly, the results echo those reported by Whittington et al. (2009), which showed that both the service user and practitioner groups preferred to avoid the use of restraint (in particular mechanical) and that the implementation of restrictive practices was favoured by older staff. These experiences of restrictive practices and their ultimate call for elimination operated in various ways in the lives of the service users and practitioners.

Balancing safety versus care was of significant importance for both service users and practitioners. Whilst it was important for practitioners to feel they could have the power to control service users and keep a safe ward environment (Wai-Tong and Lee, 2007), a safe environment meant it being aesthetically comfortable and supportive to service users (Wilson et al., 2018). This balancing act is also echoed within the wider literature, where nurse practitioners agonise between constant risk assessment and trying to provide trauma-informed care for service users (Muir-Cochrane et al., 2018). The meaning and emotional experiences of restrictive practices highlighted the complexity around the feelings towards restrictive practices from both the service users and practitioners. Whilst practitioners and service users could agree on restrictive practice intervention not being ideal, the practitioners voice was loud when it came to believing that it was a necessary tool for managing disruptive behaviours (Muir-Cochrane, 1996). The findings of the sub-theme physical and interpersonal environment are also supported by the observations of Gerace and Muir-Cochrane (2019) on empathy during conflict and the complexities of the practitioner-service user relationship.

The need for alternatives to restrictive practices were explored by both mental health service users and practitioner groups within the literature. As Kontio et al. (2012) suggested, service users' suggestions were centred in empathetic intervention and focused on communication and meaningful activities. Similarly, the practitioners viewed the need to reduce restrictive practices and their harmful consequences (Vedana et al., 2018). The challenges for transforming practice and the path to eliminating restrictive practices is no mean feat. Complexities such as staffing concerns and appropriate support for service users (Wharewera-Mika et al., 2016) were highlighted as some examples to impeding their eradication.

Re-traumatisation and dehumanisation as a result of restrictive practices and the consequential dehumanisation and power dynamics was significantly discussed by service users. Reports of powerlessness and helplessness (Chien et al., 2005) are not too uncommon and the debate about restrictive practices being a lack of respect for basic human dignity (Mayers et al., 2010) was further supported within this synthesis. The professional competencies and varying experiences of practitioners was detailed by means of exploring the factors that influence the practitioners decision-making process to implement restrictive practices. Furthermore, the meaning of quality care and practitioner support also highlighted how organisational factors are detrimental in the consideration of eliminating restrictive practices from mental health services (Bachmann et al., 2022).

This qualitative evidence synthesis is one of the first of its kind to evaluate the perceptions and experiences of both the mental healthcare service users and practitioners of the restrictive practices of both seclusion and restraint (Butterworth et al., 2022). Whilst Butterworth et al. (2022) included themes similar to this qualitative evidence synthesis, such as the psychological impact of restrictive practices, this current synthesis takes the conversation one step further by discussing theme's such as the meaning of quality of care and practitioner support.

The findings from this synthesis are explicit in detailing the different perceptions and experiences of both practitioners and services users, yet the underlying themes that link both sample groups are their basic need for safety, respect and autonomy. Whilst the practitioners detail safety in terms of their work place rights, the service users are echoing these sentiments in terms of their right to recovery orientated care (Kidd et al., 2015). The unique themes that are established in this synthesis are imperative in conveying the overwhelming need for collaborative action to promote better acute mental health care services for all impacted.

4.1. Limitations and future research directions

While this synthesis provided insight into the experiences of restrictive practices from the perspective of the service users and practitioners, it cannot be extrapolated to the full breadth of service users' and practitioners lived experiences across all population groups. Repeating the synthesis with a broader set of inclusion criteria is suggested to explore strategies for restrictive practice elimination in greater depth.

In this synthesis, it was not possible to capture all mental health practitioner perspectives as often only the perspectives of nurses and physicians were explored in the articles provided. For this reason, our findings cannot necessarily be generalised to reflect all mental health practitioners' experiences (i.e., social workers and occupational therapists). This would thus suggest the need for further research of the perspectives and experiences of restrictive practices and their proposed elimination in such demographics.

The reduction and elimination of restrictive practices has been a focus of research for a long time with sadly little effective change. Along with the importance on elevating lived experience and including service user and practitioner voice, efforts are needed to combine lived experience with a system thinking approach to better understand this complex problem. It is suggested that this co-design methodology would be an efficacious way to generate effective and sustainable solutions.

The strengths of this synthesis include the inclusion of both service users and practitioner lived experiences and their juxtaposition in this research design. Future research would be recommended to further explore their experiences and perceptions for eliminating restrictive practices from mental health services.

5. Conclusion

Restrictive practices and their elimination from mental health services is a sensitive topic with multifaceted complexity. This article is the first to present the qualitative experiences of mental health service users and practitioners and their perceptions and experiences of restrictive practices (seclusion and physical/mechanical restraint) and their ultimate elimination published in the literature. This synthesis provides insight into the many layered and often uncomfortable nature of restrictive practices which pervade many of the world's mental health facilities.

Ethics approval

Not applicable.

Funding

SLB is supported by a Higher Degree by Research Scholarship provided by Deakin University and Barwon Health. The opinions, analysis, and conclusions in this paper are those of the authors and should not be attributed to the funding bodies.

CRediT authorship contribution statement

S.L. Bennetts: Conceptualization, Data curation, Formal analysis, Investigation, Methodology, Project administration, Validation, Visualization, Writing – original draft, Writing – review & editing. **G. Pepin:** Supervision, Writing – original draft. **S. Moylan:** Supervision, Writing – original draft. **R. Carolin:** Supervision, Writing – original draft. **J.J. Lucas:** Conceptualization, Formal analysis, Supervision, Validation, Writing – original draft, Writing – review & editing.

Declaration of competing interest

The authors declare that they have no known competing financial interests or personal relationships that could have appeared to influence the work reported in this paper.

Acknowledgements

The author group would like to acknowledge the contribution of Miss Rachel West, Liaison Librarian (https://www.deakin.edu.au/about-deakin/people/rachel-west), for their review of the electronic search strategy.

The author group would also like to acknowledge Dr James McClure for their guidance on the terminology around mental health service user.

Appendix A. Supplementary data

Supplementary data to this article can be found online at https://doi.org/10.1016/j.ssmmh.2024.100305.

References

Abuse, S., Administration, M.H.S., 2011. The Business Case for Preventing and Reducing Restraint and Seclusion Use. HHS Publication No.(SMA, pp. 11–4632.

Allikmets, S., Marshall, C., Murad, O., Gupta, K., 2020. Seclusion: a patient perspective. Issues Ment. Health Nurs. 41 (8), 723–735.

Aluh, D.O., Ayilara, O., Onu, J.U., Pedrosa, B., Silva, M., Grigaite, U., Santos-Dias, M., Cardoso, G., Caldas-de-Almeida, J.M., 2023. Use of Coercion in mental healthcare services in Nigeria: service providers perspective. J. Ment. Health. https://doi.org/10.1080/09638237.2023.2182426.

Australian Insititute of Health and Welfare, 2022. Restrictive Practices in Mental Health Care. https://www.aihw.gov.au/reports/mental-health-services/mental-health-services-in-australia/data?page=1.

Bachmann, L., Vatne, S., Mundal, I.P., 2022. Safeguarding patients while implementing mechanical restraints: a qualitative study of nurses and ward staff's perceptions and assessment. J. Clin. Nurs. https://doi.org/10.1111/jocn.16249.

Bigwood, S., Crowe, M., 2008. 'It's part of the job, but it spoils the job': a phenomenological study of physical restraint. Int. J. Ment. Health Nurs. 17 (3), 215–222. https://doi.org/10.1111/j.1447-0349.2008.00526.x.

Bonner, G., Lowe, T., Rawcliffe, D., Wellman, N., 2002. Trauma for all: a pilot study of the subjective experience of physical restraint for mental health inpatients and staff in the UK. J. Psychiatr. Ment. Health Nurs. 9 (4), 465–473. https://doi.org/10.1046/j.1365-2850.2002.00504.x.

Brophy, L.M., Roper, C.E., Hamilton, B.E., Tellez, J.J., McSherry, B.M., 2016. Consumers and their supporters' perspectives on poor practice and the use of seclusion and restraint in mental health settings: results from Australian focus groups. Int. J. Ment. Health Syst. 10 https://doi.org/10.1186/s13033-016-0038-x.

Butler, A., Hall, H., Copnell, B., 2016. A guide to writing a qualitative systematic review protocol to enhance evidence-based practice in nursing and health care. Worldviews Evid. Based Nurs. 13 (3), 241–249.

Butterworth, H., Wood, L., Rowe, S., 2022. Patients' and staff members' experiences of restrictive practices in acute mental health in-patient settings: systematic review and thematic synthesis. BJPsych Open 8 (6), e178. https://doi.org/10.1192/bjo.2022.574.

Byrne, L., Stratford, A., Davidson, L., 2018. The global need for lived experience leadership. Psychiatr. Rehabil. J. 41 (1), 76.

Care, C.D.o.H.a.A., 2000. National mental health report 2000: Sixth annual report. In: Changes in Australia's Mental Health Services under the First National Mental Health Plan of the National Mental Health Strategy.

Carroll, C., 2017. Qualitative evidence synthesis to improve implementation of clinical guidelines. BMJ 356, j80. https://doi.org/10.1136/bmj.j80.

Carroll, C., Booth, A., Lloyd-Jones, M., 2012. Should we exclude inadequately reported studies from qualitative systematic reviews? An evaluation of sensitivity analyses in two case study reviews. Qual. Health Res. 22 (10), 1425–1434.

Chien, W.-T., Chan, C.W.H., Lam, L.-W., Kam, C.W., 2005. Psychiatric inpatients' perceptions of positive and negative aspects of physical restraint. Patient Educ. Counsel. 59 (1), 80–86. https://doi.org/10.1016/j.pec.2004.10.003.

Cooke, A., Smith, D., Booth, A., 2012. Beyone PICO: the SPIDER tool for qualitative evidence synthesis. Qual. Health Res. 22 (10), 1435–1443.

Critical Appraisal Skills Programme [CASP], 2018. CASP (Qualitative) checklist, Vol. 2021.

Curie, C.G., 2005. Special Section on seclusion and restraint: Commentary: SAMHSA's Commitment to eliminating the use of seclusion and restraint. Psychiatr. Serv. 56 (9), 1139–1140.

Cusack, P., Cusack, F.P., McAndrew, S., McKeown, M., Duxbury, J., 2018. An integrative review exploring the physical and psychological harm inherent in using restraint in mental health inpatient settings. Int. J. Ment. Health Nurs. 27 (3), 1162–1176.

Department of Health Victoria, 2013. Reducing Restrictive Interventions: Literature Review. Retrieved from reducing-restrictive-interventions-literature-review-2013-pdf.pdf.

Dixon-Woods, M., Cavers, D., Agarwal, S., Annandale, E., Arthur, A., Harvey, J., Smith, L., 2006. Conducting a critical interpretive synthesis of the literature on access to healthcare by vulnerable groups. BMC Med. Res. Methodol. 6 (1), 1–13.

Dyck, E., 2013. Spaced-out in Saskatchewan: Modernism, anti-psychiatry, and Deinstitutionalization, 1950-1968. Bull. Hist. Med. 84 (4), 640–666.

Ezeobele, I.E., Malecha, A.T., Mock, A., Mackey-Godine, A., Hughes, M., 2014. Patients' lived seclusion experience in acute psychaitric hospital in the United States: a qualitative study. J. Psychiatr. Ment. Health Nurs. 21, 303–312.

Fereidooni Moghadam, M., Fallahi Khoshknab, M., Pazargadi, M., 2014. Psychiatric nurses' perceptions about physical restraint; A qualitative study. International journal of community based nursing and midwifery 2 (1), 20–30. https://ezproxy.deakin.edu.au/login?url=https://search.ebscohost.com/login.aspx?direct=true&AuthType=ip,sso&db=mdc&AN=25349842&site=ehost-live&scope=site.

Fletcher, J., Spittal, M., Brophy, L., 2017. Outcomes of the Victorian Safewards trial in 13 wards: impact on seclusion rates and fidelity measurement. Int. J. Ment. Health Nurs. 26, 461–471.

Gerace, A., Muir-Cochrane, E., 2019. Perceptions of nurses working with psychiatric consumers regarding the elimination of seclusion and restraint in psychiatric inpatient settings and emergency departments: an Australian survey. Int. J. Ment. Health Nurs. 28 (1), 209–225.

Gerrand, V., 2005. Can deinstitutionalisation work? Mental health reform from 1993 to 1998 in Victoria, Australia. Health Sociol. Rev.: The Journal of the Health Section of the Australian Sociological Association 14 (3), 255–271.

Gooding, P., 2016. From deinstitutionalisation to consumer empowerment: mental health policy, neoliberal restructuring and the closure of the 'Big bins' in Victoria. Health Sociol. Rev. 25 (1), 33–47.

Hem, M.H., Gjerberg, E., Husum, T.L., Pedersen, R., 2018. Ethical challenges when using coercion in mental healthcare: a systematic literature review. Nurs. Ethics 25 (1), 92–110.

Hernandez, A., Riahi, S., Stuckley, M.I., Mildon, B.A., Klassen, P.E., 2017. Multidimensional approach to restraint minimisation: they journey of a specialised mental health organisation. Int. J. Ment. Health Nurs. 26, 482–490.

Hoff, P., 2015. Mesures de contrainte en médecine. Bull. Med. Suisses 96 (22).

Holmes, D., Kennedy, S.L., Perron, A., 2004. The mentally ill and social exclusion: a critical examination of the use of seclusion from the patient's perspective. Issues Ment. Health Nurs. 25 (6), 559–578. https://doi.org/10.1080/01612840490472101.

Jacob, J.D., Holmes, D., Rioux, D., Corneau, P., MacPhee, C., 2019. Convergence and divergence: an analysis of mechanical restraints. Nurs. Ethics 26 (4), 1009–1026. https://doi.org/10.1177/0969733017736923.

Kidd, S., Kenny, A., McKinstry, C., 2015. Exploring the meaning of recovery-oriented care: an action-research study. Int. J. Ment. Health Nurs. 24 (1), 38–48.

Kodua, M., Duxbury, J., Eboh, W.O., Asztalos, L., Tweneboa, J., 2023. Healthcare staff's experiences of using manual physical restraint: a meta-synthesis review. Nurs. Health Sci. 25 (3), 271–289.

Kontio, R., Joffe, G., Putkonen, H., Kuosmanen, L., Hane, K., Holi, M., Välimäki, M., 2012. Seclusion and restraint in psychiatry: patients' experiences and practical suggestions on how to improve practices and use alternatives. Psychiatr. Care 48 (1), 16–24. https://doi.org/10.1111/j.1744-6163.2010.00301.x.

Kontio, R., Välimaäki, M., Putkonen, H., Kuosmanen, L., Scott, A., Joffe, G., 2010. Patient restrictions: are there ethical alternatives to seclusion and restraint? Nurs. Ethics 17 (1), 65–76. https://doi.org/10.1177/0969733009350140.

Lanthén, K., Rask, M., Sunnqvist, C., 2015. Psychiatric patients experiences with mechanical restraints: an interview study. Psychiatry journal, 748392. https://doi.org/10.1155/2015/748392, 2015.

Larue, C., Dumais, A., Boyer, R., Goulet, M.-H., Bonin, J.-P., Baba, N., 2013. The experience of seclusion and restraint in psychiatric settings: perspectives of patients. Issues Ment. Health Nurs. 34 (5), 317–324.

Larue, C., Dumais, A., Drapeau, A., Ménard, G., Goulet, M.-H., 2010. Nursing practices recorded in reports of episodes of seclusion. Issues Ment. Health Nurs. 31 (12), 785–792. https://doi.org/10.3109/01612840.2010.520102.

Larue, C., Piat, M., Racine, H., Ménard, G., Goulet, M., 2010. The nursing decision making process in seclusion episodes in a psychiatric facility. Issues Ment. Health Nurs. 31 (3), 208–215. https://doi.org/10.3109/01612840903131800.

LeBel, I.B., Duxbury, J., Putkomen, A., Sprague, T., Rae, C., Sharpe, J., 2014. Multinational experiences in reducing and preventing the use of restraint and seclusion. J. Psychosoc. Nurs. Ment. Health Serv. 52, 22–29.

LeBel, J., Goldstein, R., 2005. Special section on seclusion and restraint: the economic cost of using restraint and the value added by restraint reduction or elimination. Psychiatr. Serv. 56 (9), 1109–1114.

Ling, S., Cleverley, K., Perivolaris, A., 2015. Understanding mental health service user experiences of restraint through debriefing: a qualitative analysis. Can. J. Psychiatr. 60 (9), 386–392.
Lucas, J.J., Bouchoucha, S.L., Afrouz, R., Reed, K., Brennan-Olsen, S.L., 2022. LGBTQ+ loss and grief in a Cis-Heteronomative pandemic: a qualitative evidence synthesis of the COVID-19 literature. Qual. Health Res. 1–16.
Manderius, C., Clintstahl, K., Sjostrom, K., Ormon, K., 2023. The psychiatric mental health nurses' ethical considerations regarding the use of coercive measures - a qualitative interview study. BMC Nurs. 22 (23).
Manzano-Bort, Y., Mir-Abellán, R., Via-Clavero, G., Llopis-Cañameras, J., Escuté-Amat, M., Falcó-Pegueroles, A., 2021. Experience of mental health nurses regarding mechanical restraint in patients with psychomotor agitation: a qualitative study. J. Clin. Nurs. https://doi.org/10.1111/jocn.16027.
Martello, M., Doronina, O., Perillo, A., La Riccia, P., Lavoie-Tremblay, M., 2018. Nurses' perceptions of engaging with patients to reduce restrictive practices in an inpatient psychiatric unit. Health Care Manag. 37 (4), 342–353. https://doi.org/10.1097/HCM.0000000000000235.
Mayers, P., Keet, N., Winkler, G., Flisher, A.J., 2010. Mental health service users? Perceptions and experiences of sedation, seclusion and restraint. Int. J. Soc. Psychiatr. 56 (1), 60–73. https://doi.org/10.1177/0020764008098293.
McCain, M., Kornegay, K., 2005. Behavioral health restraint: the experience and beliefs of seasoned psychiatric nurses. J. Nurses Staff Dev. 21 (5), 236–242. https://ezproxy.deakin.edu.au/login?url=https://search.ebscohost.com/login.aspx?direct=true&AuthType=ip,sso&db=ccm&AN=106379676&site=ehost-live&scope=site.
Meadows, G., Singh, B., 2003. 'Victoria on the move': mental health services in a decade of transition 1992-2002. Australian Psychiatry 11 (1), 62–67.
Meehan, T., Vermeer, C., Windsor, C., 2000. Patients' perceptions of seclusion: a qualitative investigation. J. Adv. Nurs. 31 (2), 370–377.
Mooney, M., Kanyeredzi, A., 2021. 'You get this conflict between you as a person and you in your role that changes you': a thematic analysis of how inpatient psychiatric healthcare staff in the UK experience restraint, seclusion, and other restrictive practices. Int. J. Ment. Health Nurs. 30 (6), 1703–1712.
Moran, A., Cocoman, A., Scott, P.A., Mathews, A., Staniulene, V., Valimaki, M., 2009. Restraint and seclusion: a distressing treatment option? Journal of Psychiatric & Mental Health Nursing (Wiley-Blackwell 16 (7), 599–605. https://doi.org/10.1111/j.1365-2850.2009.01419.x.
McSherry, B., Maker, Y., 2020. Restrictive Practices in Health Care and Disability Settings: Legal, Policy and Practical Responses. Routledge.
Muir-Cochrane, E., 1996. An investigation into nurses' perceptions of secluding patients on closed psychiatric wards. J. Adv. Nurs. 23 (3), 555–563. https://doi.org/10.1111/j.1365-2648.1996.tb00019.x.
Muir-Cochrane, E., Grimmer, K., Gerace, A., Bastiampillai, T., Oster, C., 2020. Prevalence of the use of chemical restraint in the management of challenging behaviours associated with adult mental health conditions: A meta-synthesis. J. Psychiatr. Ment. Health Nurs. 27 (4), 425–445. https://doi.org/10.1111/jpm.12585.
Muir-Cochrane, E., Harrison, B., 1996. Therapeutic interventions associated with seclusion of acutely disturbed individuals. J. Psychiatr. Ment. Health Nurs. 3 (5), 319–325. https://doi.org/10.1111/j.1365-2850.1996.tb00132.x.
Muir-Cochrane, E., O'Kane, D., Oster, C., 2018. Fear and blame in mental health nurses' accounts of restrictive practices: implications for the elimination of seclusion and restraint. Int. J. Ment. Health Nurs. 27 (5), 1511–1521. https://doi.org/10.1111/inm.12451.
Muir-Cochrane, E.C., Baird, J., McCann, T.V., 2015. Nurses' experiences of restraint and seclusion use in short-stay acute old age psychiatry inpatient units: a qualitative study. J. Psychiatr. Ment. Health Nurs. 22 (2), 109–115. https://doi.org/10.1111/jpm.12189.
Muir-Cochrane, E., 2018. On restrictive practices: care and culture. J. Psychiatr. Ment. Health Nurs. 25 (8), 451–452. https://doi.org/10.1111/jpm.12492.
Olofsson, B., Norberg, A., 2001. Experiences of coercion in psychiatric care as narrated by patients, nurses and physicians. J. Adv. Nurs. 33 (1), 89–97. https://doi.org/10.1046/j.1365-2648.2001.01641.x.
Page, M.J., McKenzie, J.E., Bossuyt, P.M., Boutron, I., Hoffman, T.C., Mulrow, C.D., 2021. The PRISMA 2020 Statement: an Updated Guideline for Reporting Systematic Reviews, p. 372. BMJ (Clinical research ed.).
Perez-Toribio, A., Moreno-Poyato, A.R., Roldán-Merino, J.F., Nash, M., 2021. Spanish mental health nurses' experiences of mechanical restraint: a qualitative descriptive study. J. Psychiatr. Ment. Health Nurs. 29, 688–697.
Perkins, R., Rinaldi, M., Hardisty, J., 2010. Harnessing the expertise of experience: increasing access to employment within mental health services for people who have themselves experienced mental health problems. Divers. Health Care 7 (1).
Power, T., Baker, A., Jackson, D., 2020. 'Only ever as a last resort': mental health nurses' experiences of restrictive practices. Ment. Health Nurs. 29 (4), 674–684.
Putkonen, H., Vollm, B., 2007. Compulsory psychiatric detention and treatment in Finland. Psychiatr. Bull. 31 (3), 101–103.
Sambrano, R., Cox, L., 2013. 'I sang Amazing Grace for about 3 hours that day': understanding Indigenous Australians' experience of seclusion. Int. J. Ment. Health Nurs. 22 (6), 522–531. https://ezproxy.deakin.edu.au/login?url=https://search.ebscohost.com/login.aspx?direct=true&AuthType=ip,sso&db=psyh&AN=2013-35355-007&site=ehost-live&scope=site.
Scholes, A., Price, O., Berry, K., 2022. Women's experiences of restrictive interventions within inpatient mental health services: a qualitative investigation. Int. J. Ment. Health Nurs. 31 (2), 379–389. https://doi.org/10.1111/inm.12966.
Sequeira, H., Halstead, S., 2002. Control and restraint in the UK: service user perspectives. Br. J. Forensic Pract. 4 (1), 9–18. https://doi.org/10.1108/14636646200200003.
Sequeira, H., Halstead, S., 2004. The psychological effects on nursing staff of administering physical restraint in a secure psychiatric hospital: 'When I go home, it's then that I think about it. Br. J. Forensic Pract. 6 (1), 3–15. https://doi.org/10.1108/14636646200400002.
Smith, G.M., Ashbridge, D.M., Davis, R.H., Steinmetz, W., 2015. Correlation between reduction of seclusion and restraint and assaults by patients in Pennsylvania's State Hospitals. Psychiatr. Serv. 66, 303–309.
Smith, S.B., 1995. Restraints: retraumatization for rape victims? J. Psychosoc. Nurs. Ment. Health Serv. 33 (7), 23–28. https://doi.org/10.3928/0279-3695-19950701-06.
State of Victoria, 2021. Royal Commission into Victoria's Mental Health System: Final Report (PP No 202, Session 2018-2021 (document 1 of 6), Issue.
Steinert, T., Lepping, P., Bernhardsgrütter, R., Conca, A., Hatling, T., Janssen, W., Keski-Valkama, A., Mayoral, F., Whittington, R., 2010. Incidence of seclusion and restraint in psychiatric hospitals: a literature review and survey of international trends. Soc. Psychiatr. Psychiatr. Epidemiol. 45, 889–897.
Thomas, J., Harden, A., 2008. Methods for the thematic synthesis of qualitative research in systematic reviews. BMC Med. Res. Methodol. 8 (1), 1–10.
VanDerNagel, J.E.L., Tuts, K.P., Hoekstra, T., Noorthoorn, E.O., 2009. Seclusion: the perspective of nurses. Int. J. Law Psychiatr. 32 (6), 408–412. https://doi.org/10.1016/j.ijlp.2009.09.011.
Varpula, J., Välimäki, M., Lantta, T., Berg, J., Lahti, M., 2020. Nurses' perceptions of risks for occupational hazards in patient seclusion and restraint practices in psychiatric inpatient care: a focus group study. Int. J. Ment. Health Nurs. https://doi.org/10.1111/inm.12705.
Vedana, K.G.G., da Silva, D.M., Ventura, C.A.A., Giacon, B.C.C., Zanetti, A.C.G., Miasso, A.I., Borges, T.L., 2018. Physical and mechanical restraint in psychiatric units: perceptions and experiences of nursing staff. Arch. Psychiatr. Nurs. 32 (3), 367–372. https://doi.org/10.1016/j.apnu.2017.11.027.
Wai-Tong, C., Lee, I.Y.M., 2007. Psychiatric nurses' knowledge and attitudes toward the use of physical restraint on older patients in psychiatric wards. Int. J. Mult. Res. Approaches 1 (1), 52–71. https://doi.org/10.5172/mra.455.1.1.52.
Wharewera-Mika, J., Cooper, E., Wiki, N., Field, T., Haitana, J., Toko, M., Edwards, E., McKenna, B., 2016. Strategies to reduce the use of seclusion with tāngata whai I te ora (Māori mental health service users). Int. J. Ment. Health Nurs. 25 (3), 258–265. https://doi.org/10.1111/inm.12219.
Wieman, D.A., Carmacho-Gonsalves, T., Huckson, K.A., Leff, S., 2014. Multisite evaluation of an evidenced-based practise to reduce seclusion and restrain in pscychiatric patient facilities. Psychiatr. Serv. 65, 345–351.
Wilson, C., Rouse, L., Rae, S., Kar Ray, M., 2018. Mental health inpatients' and staff members' suggestions for reducing physical restraint: a qualitative study. J. Psychiatr. Ment. Health Nurs. 25 (3), 188–200. https://doi.org/10.1111/jpm.12453.
Wong, A.H., Ray, J.M., Eixenberger, C., Crispino, L.J., Parker, J.B., Rosenberg, A., Robinson, L., McVaney, C., Iennaco, J.D., Bernstein, S.L., Yonkers, K.A., Pavlo, A.J., 2022. Qualitative study of patient experiences and care observations during agitation events in the emergency department: implications for systems-based practice. BMJ Open 12 (5), e059876. https://doi.org/10.1136/bmjopen-2021-059876.
Whittington, R., Bowers, L., Nolan, P., Simpson, A., Neil, L., 2009. Approval ratings of inpatient coercive interventions in a national sample of mental health service users and staff in England. Psychiatr. Serv. 60 (6), 792–798. https://doi.org/10.1176/appi.ps.60.6.792.
Wynaden, D., Chapman, R., McGowan, S., Holmes, C., Ash, P., Boschman, A., 2002. Through the eye of the beholder: to seclude or not to seclude. Int. J. Ment. Health Nurs. 11 (4), 260–268. https://doi.org/10.1046/j.1440-0979.2002.00257.x.
Zayfert, C., 2012. Cognitive behavioral conceptualization of retraumatization. Retraumatization: Assessment, treatment, and prevention 9–32.
Zimmer, L., 2006. Qualitative meta-synthesis: a question of dialoguing with texts. J. Adv. Nurs. 53 (3), 311–318.